INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Fortnightly cetuximab + FOLFIRI

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
14 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00585
Version 5b

Cetuximab and FOLFIRI on Day 1 every 14 days; a 46-hour fluorouracil pump ends around Day 3. No Day 8 infusion.

Treatment stages

01 **A 14-day reference cycle**
Clinic → home → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

Fortnightly cetuximab + FOLFIRI
National Cancer Control Programme (NCCP) 00585

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Report eye pain, blurred vision, blistering or infected skin promptly. Call for line pain, swelling, leakage or unexpected pump progress.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How your medicines work

The aim is to control advanced bowel cancer and symptoms. Treatment continues while it is helping and side effects remain acceptable; your team reviews the plan with you.

EGFR-TARGETED ANTIBODY

Cetuximab

Erbitux



Blocks growth messages at the cell surface

Cetuximab attaches to EGFR on the outside of cells and blocks some growth messages. In this bowel-cancer plan, validated extended RAS testing helps identify whether treatment is suitable. Healthy skin also uses EGFR, so skin and nail effects need early care.

EGFR receives a growth signal → Cetuximab blocks the receptor → Some growth signals are reduced

What to watch for

Report rash or sore nails early. Tell the nurse immediately about infusion symptoms. Eye pain, new breathing problems or severe diarrhoea need prompt advice. Blood tests check minerals during treatment and for two months afterwards.

TOPOISOMERASE I CHEMOTHERAPY

Irinotecan

Campto



Interrupts deoxyribonucleic acid (the cell’s genetic material) (DNA) copying

The body converts irinotecan into SN-38. This traps an enzyme called topoisomerase I while DNA is being copied, causing damage that can stop cancer cells dividing. Normal bowel and blood-forming cells can also be affected.

Irinotecan becomes SN-38 → DNA-repair enzyme is trapped → Dividing cells are damaged

What to watch for

Tell the nurse immediately about cramps, sweating or diarrhoea during the infusion. Diarrhoea starting later needs the separate written treatment and contact plan.

CHEMOTHERAPY MODULATOR

Folinic acid

Calcium leucovorin



Strengthens fluorouracil action

Folinic acid helps fluorouracil remain bound to an enzyme needed to make DNA. In this regimen it increases fluorouracil activity and can also increase its unwanted effects. It is not a vitamin supplement to start yourself.

Folinic acid → Stronger enzyme binding → Fluorouracil effect increases

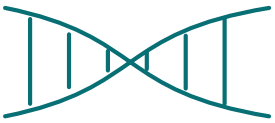
What to watch for

The combined treatment can increase mouth soreness and diarrhoea. Report these promptly. Rare allergic reactions can occur.

CHEMOTHERAPY

Fluorouracil

5-FU



Disrupts cell building blocks

Fluorouracil interferes with the production and use of DNA and RNA, the instructions cells need to grow. The infusion extends exposure over approximately two days.

Drug enters cell → DNA / RNA disrupted → Growth slows

What to watch for

Diarrhoea, mouth sores and skin soreness may become serious. Chest pain during the pump infusion needs emergency assessment. Unexpected severe early toxicity can occur even after dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD) testing.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

The setting changes what benefit means.

Why this option?

This combination is used for RAS wild-type bowel cancer that has spread. Your team explains the aim, alternatives and how benefit will be reviewed.

A plan that can change

This regimen is used to control advanced bowel cancer and symptoms. Ask how benefit will be assessed and whether any local treatment is part of your individual plan. Your preferences and priorities matter.

CRYSTAL: extended RAS analysis

Studies can help explain why a treatment is offered. The schedule, previous medicines and tumour tests affect how well the evidence fits you.

In a selected bowel-cancer trial, adding cetuximab to FOLFIRI helped more tumours shrink and delayed cancer growth; patients also lived longer as a group. Your cancer biology, previous treatment and health affect the likely benefit. These results cannot predict your own outcome. The main trial used weekly cetuximab, so it is supporting evidence for this fortnightly schedule.

CHAPTER 02

Your schedule, tests and appointments

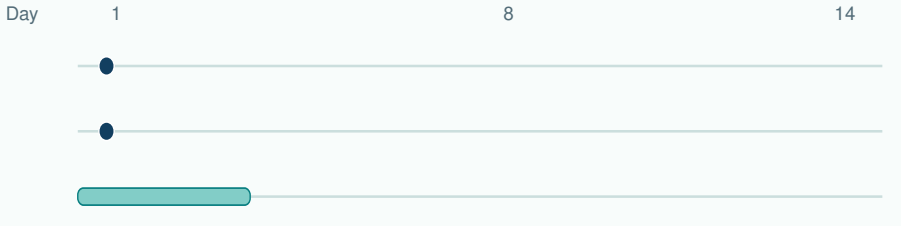
IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment, recovery and review

Fortnightly cetuximab + FOLFIRI

14-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

CLINIC → HOME → REVIEW

A 14-day reference cycle

D1 D3

Cetuximab and FOLFIRI start on Day 1, followed by a 46-hour fluorouracil infusion with planned disconnection around Day 3. Repeat every 14 days while beneficial and tolerable. No weekly Day 8 cetuximab is scheduled.

A repeating reference cycle sits within a longer treatment plan. The oncology team chooses the number of cycles and confirms changes; no patient dates are required here.

BASELINE → TREATMENT → FOLLOW-UP

When checks happen

The columns show order, not equal periods. There is no fixed number of cycles for these plans. Agree who books each check, when results are reviewed and whom to contact if a check is missed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00585 v5b · current official regimen checked 5 November 2026 · EMA: Erbitux product information and patient leaflet

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Fortnightly cetuximab + FOLFIRI: treatment and checks

Advanced / metastatic colorectal cancer

Cetuximab and FOLFIRI start on Day 1, followed by a 46-hour fluorouracil infusion with planned disconnection around Day 3. Repeat every 14 days while beneficial and tolerable. No weekly Day 8 cetuximab is scheduled.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Blood count, kidney and liver NCCP 00585	Baseline blood count, kidney and liver profile	Review results and symptoms before treatment	Blood count, kidney and liver profile before every cycle; weekly blood counts when delayed for low counts	Repeat sooner if unwell or results need follow-up	Follow unresolved abnormalities after treatment
Magnesium, calcium and potassium NCCP 00585	Check mineral levels before starting	Repeat periodically during treatment	Keep blood-test and replacement appointments	More checks may be needed if levels fall or diarrhoea develops	Monthly checks for two months after the last antibody dose
Skin, nails, eyes and breathing NCCP 00585	Discuss existing skin, eye or lung problems	Start the prescribed skin-care plan; report reactions early	Review skin, nails, diarrhoea and new symptoms	Eye pain, worsening breathing or severe rash needs prompt assessment	Continue care until symptoms and mineral levels settle
Bowel symptoms and hydration NCCP 00585	Have a written diarrhoea plan and emergency contact	Tell the nurse about cramps, sweating or diarrhoea during infusion	Delayed diarrhoea can start after the first day; act using your written plan	Fever, reduced urine, severe pain or inability to drink needs urgent advice	Restart decisions require recovery and team review
Fluorouracil safety and line NCCP 00585	Confirm the DPD test and safe line/ pump arrangements	Check mouth, bowel and pump symptoms early	Review line, pump and previous-cycle toxicity	Report chest pain or unexpectedly severe early side effects immediately	Confirm disconnection and ongoing line care

Care / check	Before starting	Early treatment	Repeated treatment	Longer-term review	Pause / completion
Cancer response and treatment choices NCCP 00585	Record baseline disease and agree review plan	Bring forward review if symptoms worsen	Scans and other tests follow your consultant's treatment plan	Discuss benefit, side effects and whether the goal has changed	Agree continuation, a break, another treatment or supportive care
Conditional heart assessment NCCP 00585	electrocardiogram (ECG) if cardiac function is compromised	Report new chest pain or breathing change	Emergency assessment for chest pain, collapse or severe breathlessness	Additional cardiac tests when clinically indicated	Follow any identified problem; no routine fixed echo series
international normalised ratio (blood-clotting test) (INR) if taking warfarin NCCP 00585	Tell the team about warfarin	Check INR as clinically indicated	Report bleeding and major changes in food intake	Warfarin doses are adjusted by the prescribing team	Continue the individually agreed anticoagulation plan

What each check is for

1 A 14-day reference cycle

Clinic → home → review

The aim is to control advanced bowel cancer and symptoms. Treatment continues while it is helping and side effects remain acceptable; your team reviews the plan with you.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your port and treatment pump

From preparing the line to coming home after disconnection. Your team arranges the appointments and shows you what to do.

1 · Before treatment

Choose and prepare the line

The team explains whether a port or peripherally inserted central catheter (PICC) suits your treatment. You receive instructions for the insertion appointment.

2 · Insertion and recovery

A separate appointment may be needed

For a port, numbing medicine is used and a small device is placed under the skin. Ask about sedation, transport and wound care.

3 · Day 1

The nurse connects your pump

After the day-ward medicines, the nurse checks and connects the fluorouracil pump. Ask for a demonstration before leaving.

4 · At home

Treatment continues for about 46 hours

Carry the pump as shown. Keep the dressing dry and protect the line from pulling, heat and damage.

5 · As arranged

Return for disconnection

A trained professional checks the infusion, disconnects the pump and cares for the line. The appointment is based on connection time.

6 · Between cycles

The access device usually stays

A port needle can be removed while the port stays under the skin. A PICC remains in the arm with ongoing dressing and line care.

Port or PICC: what is the difference?

A port is a small chamber beneath the skin, usually on the chest, attached to a tube ending in a large vein. A special needle goes through the skin into the port when it is used. A PICC enters a vein in the upper arm; a short section remains outside under a dressing. Both can deliver a home infusion. Your team helps choose the device and teaches its care.

Before insertion and during recovery

Tell the team about allergies, blood thinners, infections and any other cancer medicines. Follow the instructions about eating, drinking and transport; do not stop medicines yourself. Port placement uses local anaesthetic, sometimes with sedation or general anaesthetic. The team explains the procedure and risks, including infection, bleeding, clots and insertion complications. Ask how to manage discomfort, protect the wound and resume activity. Report increasing pain, redness, swelling, discharge or fever.

Connection: what the nurse does

The nurse checks the line and treatment, secures the connection and dressing, and starts the pump. Before leaving, you should have the disconnection location and time, contact numbers including out of hours, and the written spill instructions. Ask to be shown what normal pump progress looks like. You do not need to calculate a rate.

Carrying it, sleeping and washing

Use the supplied carrier and the position recommended for your pump. Keep tubing free from pulling and kinks. Ask how to place the pump safely for sleep and how to protect the line and dressing when washing. Do not immerse it or swim. Avoid direct heat and cold. An elastomeric pump has a balloon that shrinks slowly and may have no alarm; an electronic pump has different instructions. Follow the instructions for the device you actually receive.

Disconnection is not port removal

Keep the appointment even if the pump looks empty, unless the team changes the plan. Staff check the pump and line and arrange safe disposal. After flushing and other line care as required, the port needle may be removed and a small dressing applied. The implanted port stays in place until a separate removal procedure is arranged. A PICC remains in place and needs scheduled dressing and flushing care.

Leakage, disconnection or unexpected pump progress

Call the team immediately if the pump leaks, is damaged, finishes unexpectedly early, appears not to run, or the line disconnects. Follow the emergency or spill steps you have been specifically taught, including use of a clamp if instructed. Do not reconnect, change settings or disconnect the pump yourself. Keep other people away from leaked medicine. If medicine touches skin, rinse it off and wash with soap and water, then seek advice.

Pain, swelling or feeling unwell

Contact oncology immediately for pain or swelling around the line, a wet/loose dressing, redness, fever, shivering or suddenly feeling unwell. A painful swollen arm or neck needs urgent advice. Severe chest pain, major breathing difficulty or collapse: call 112 / 999. Do not wait for pump disconnection or the next clinic visit.

The drawings show general steps, not instructions for inserting or accessing a line. Your hospital confirms its own arrangements and the care instructions for your device.

Visits, access and care between treatments

STEP 1

Before the first visit

Confirm the exact regimen, tests, medicines, emergency number and treatment goal. Discuss which central line is suitable and arrange pump training.

STEP 2

At the treatment unit

Fortnightly cetuximab infusion time is set from the dose and safe infusion rate; the first dose may take longer. Irinotecan usually takes about 90 minutes. Blood review, preparation, premedicines and observation add time; infusion time is not the total appointment length.

STEP 3

After the infusion

The fluorouracil pump runs for about 46 hours. Confirm where and when it will be disconnected, usually around Day 3; do not disconnect it yourself unless specifically trained.

STEP 4

Between doses

Use the written symptom and support plan; do not wait for the next appointment if urgent symptoms appear.

STEP 5

The next decision

Confirm the next visit after any delay or dose change. Treatment continues only while the oncology team judges benefit and tolerability acceptable. Follow-up mineral checks continue after the final EGFR-antibody dose where applicable.

Transport and daily life

Plan transport if antihistamines, symptoms or other medicines affect driving. Ask for practical, work, nutrition or emotional support.

If treatment changes

Ask which component has been held and which visits remain. A treatment delay does not automatically cancel blood tests, line care or symptom follow-up.

Before the first treatment

Checks reduce avoidable risk.

Blood tests and medicines

Review blood counts, kidney/liver tests, symptoms, allergies and all medicines and supplements. Confirm DPD testing before fluorouracil. Kidney impairment can affect uracil-based DPD testing interpretation. Tell the team about red-meat allergy or tick bites before cetuximab because these can be associated with infusion-reaction risk.

Preparing access

Confirm port/PICC preparation, pump connection, home care and professional disconnection around Day 3.

Visits and treatment sequence

Know which steps happen in the clinic and at home.

The exact delivery plan

Cetuximab on Day 1. Day 1: irinotecan, folinic acid and fluorouracil, then a 46-hour fluorouracil pump; planned disconnection around Day 3. A repeating 14-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

Radiotherapy relationship

This NCCP regimen does not contain a routine radiotherapy course. If radiation is recommended separately, ask whether it is before, during or after these medicines.

When treatment changes or ends

Follow-up continues.

Symptoms and monitoring

Continue the cetuximab skin plan and mineral checks for two months after the last dose. Ask which medicines to continue, when to seek help and how response will be assessed.

Next decisions

The team reviews benefit, side effects and your priorities. Supportive and palliative care can help alongside active treatment. Ask for a clear next-step and contact plan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Cetuximab infusion	 **Day 1** intravenous (IV)
Day-ward chemotherapy	 **Day 1** IV
Fluorouracil pump	 **Days 1–3** About 46 hours
Skin, nails and minerals	 **Days 1–14** Can build over successive cycles
Early cramps / sweating	 **Day 1** Tell the nurse
Delayed diarrhoea	 **Days 2–14** Can occur after 24 hours
Blood counts / infection risk	 **Days 1–14** Blood tests and symptoms guide review
Energy and bowel symptoms	 **Days 1–14** Variable timing

Your planned visits: Cetuximab on Day 1. Day 1: irinotecan, folinic acid and fluorouracil, then a 46-hour fluorouracil pump; planned disconnection around Day 3. A repeating 14-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

Care continues: Continue the skin-care plan and mineral checks arranged after cetuximab. Ask which medicines to continue and when your next review is planned.

No guaranteed safe days: An infection, reaction or other urgent problem can occur at any time. The graph does not predict a personal low point.

Why these bands stay broad: NCCP defines the treatment days. Symptom bands are broad educational guidance; reactions can occur on any day, and cumulative effects may continue between cycles. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00585 v5b · current official regimen checked 5 November 2026 ↗

There are no guaranteed "safe days". Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override any timeline or diary.

Between-treatment notes

Day 1: Treatment and checks

The team reviews blood tests, symptoms and your prescription before treatment. Premedicines and observation are part of cetuximab visits. Know the contact number and the next planned visit.

Day 2–3: Pump completion

Keep the arranged professional pump-disconnection appointment. Do not adjust, reconnect or remove it yourself. Call early for symptoms; do not wait for the next appointment.

Day 4–14: Recovery and the next review

The team uses symptoms, tests and your priorities to decide what happens next. Bring questions and a symptom record if useful; this is optional.

Support around the medicines

1 Skin, nails and minerals

Start the prescribed skin-care plan early, use moisturiser and sun protection, and report painful or infected skin or nails. Keep mineral blood-test appointments during treatment and for two months after cetuximab.

2 Bowel symptoms and fluids

Irinotecan can cause early cramps/sweating during infusion and delayed diarrhoea after 24 hours. You need separate instructions for each. Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice.

3 Comfort and daily life

Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

4 The whole treatment plan

Ask how and when the team will assess benefit, which blood tests continue during a pause, and what happens after the final dose. Mineral checks continue monthly for two months after the last antibody treatment.

5 Act at the first liquid stool

Use the written diarrhoea rescue prescription supplied by your oncology team at the first liquid stool and drink the advised electrolyte-containing fluids. Contact the team early; if no plan was supplied, call immediately. Severe pain, fever, blood, inability to drink or reduced urine needs urgent assessment. Do not treat early infusion cramps or sweating as the same problem.

6 Recovery and the next dose

A delay or reduction is individual. Tell the team about ongoing side effects and previous dose changes; do not assume a calendar date or a stamped card means that treatment can restart.

Symptoms in context

Report what changed and how it affects you.

Living with treatment

Use the support available.

Pregnancy and fertility

Discuss fertility preservation and reproductive plans before treatment. Ask for the contraception and breastfeeding instructions for every medicine; the whole regimen may require longer precautions than one medicine alone.

Pauses, recovery and follow-up

Your team may pause or change one or more medicines. Do not restart or change doses yourself.

Decisions before restarting

The team checks the severity and recovery of side effects, previous episodes and the doses you actually received. A standard table dose does not automatically override a prior individual reduction.

Keep testing appointments

Blood tests may continue during a treatment pause. Keep monthly electrolyte, magnesium and calcium checks for two months after the last cetuximab dose. Scans and tumour-marker timing follow the consultant plan; these documents do not impose a universal scan interval.

The interval before chemotherapy

Cetuximab is given before chemotherapy. Product information requires at least one hour between the end of cetuximab and the start of chemotherapy. Your appointment includes preparation, premedication and observation as well as time connected to a drip.

Check every medicine and supplement

Show the team all prescribed, over-the-counter and herbal medicines before treatment and before starting anything new. Certain antiviral medicines, including brivudine and sorivudine, must not be combined with fluorouracil. Tell the team if you take warfarin or phenytoin; extra monitoring may be needed. Do not stop or change medicines yourself.

Irinotecan medicine interactions

Ask the oncology pharmacist before taking grapefruit, herbal products or any new medicine. Some medicines change irinotecan exposure. Prochlorperazine should not be taken on the same day as irinotecan; confirm your alternative prescribed nausea plan with the team. Tell the team about known Gilbert syndrome before the first treatment.

Questions worth asking

Do I need to enter dates to understand the cycle?

No. The graph explains a general pattern. Your hospital arranges appointments; the guide and its safety information are available without entering anything.

What happens if a dose is delayed?

The team checks the cause and may delay, reduce, change or stop one medicine. Ask which component changes and when you should next attend; do not restart yourself.

How will I know whether it is helping?

Scans, symptoms and clinical review assess benefit. A rash, side effect or good day alone cannot show whether the cancer is responding.

Why is RAS testing mentioned?

RAS testing selects cetuximab treatment in colorectal cancer. It is not the selection test for the separate head-and-neck radiotherapy indication. Confirm the cancer-specific plan with your team.

Try explaining the plan back: Can you explain which visits are planned, whether a pump is involved, and which symptoms mean you should call today? No dates or symptom scores are needed to use this guide.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · chemotherapy and antibody

Active medicines: Cetuximab + Irinotecan + Folinic acid + Fluorouracil

Emetogenic risk: Moderate

Medicine	Dose / route	When
Antihistamine	As prescribed in the local cetuximab premedication plan	At least one hour before cetuximab Repeat for subsequent cetuximab infusions; assess prior infusion reactions.
Dexamethasone	8 mg PO once, subject to reconciliation with the cetuximab premedication prescription	At least one hour before cetuximab on Day 1 This corticosteroid also covers the treatment-day corticosteroid part of antiemesis where appropriate. Pharmacy must reconcile route, dose and timing; do not automatically add a second 8 mg antiemetic dose.
Ondansetron	16 mg PO once	Before Day 1 chemotherapy NCIS V8 moderate-risk row; consider QT risk, low electrolytes, interactions and the individual prescription.
Dexamethasone	8 mg PO daily	Days 2 and 3 after the administered Day 1 chemotherapy NCIS V8 delayed prophylaxis. Reconcile with any other steroid prescription; re-anchor dates if treatment is delayed.
Metoclopramide	10 mg PO up to three times daily as required	Rescue nausea treatment only as prescribed NCIS V8 rescue option; check contraindications, renal function, interactions and prescribed duration.
Atropine sulphate	250 micrograms subcutaneously	Before irinotecan when prescribed; assess acute cholinergic symptoms Avoid in glaucoma. This treats/prevents the acute cholinergic syndrome; it is not the delayed-diarrhoea rescue medicine. Use caution in asthma, as specified in the NCCP acute cholinergic-syndrome advice.

Additional phase-specific support

- Antiemetic classification refers to the active treatment day. Reconcile duplicate steroid entries before prescribing.
- No automatic granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) schedule is added; use the patient's neutropenia history, risk and consultant plan.
- Cetuximab premedication and infusion observation remain necessary even on an antibody-only day.
- Start local EGFR skin care; monitor magnesium, calcium and potassium before and periodically during therapy, then monthly for two months after the final dose.
- Provide a written irinotecan delayed-diarrhoea rescue and escalation plan before treatment; distinguish it from immediate infusion-related cramps/sweating. Fever, severe pain, dehydration or diarrhoea not responding to the prescribed plan needs urgent review.

- Confirm line care, pump contact details and disconnection arrangements; report chest pain, severe early fluorouracil toxicity or pump/line problems urgently.
- Supply a written irinotecan delayed-diarrhoea rescue prescription: NCCP reference loperamide 4 mg initially, then 2 mg every 2 hours; continue for 12 hours after the last liquid stool, never longer than 48 hours at this high-dose schedule. Start advised electrolyte-containing fluids at the first liquid stool and contact the team early. Fever, dehydration, severe pain or persistent symptoms require urgent oncology review.
- Irinotecan treatment-related diarrhoea must fully resolve before restart; a table-grade threshold alone does not clear the next dose.
- If treatment is delayed for cytopenia, follow the source-specific weekly blood-count checks and maximum delay; assess the antibody and chemotherapy components separately when indicated.
- EMA Erbitux section4.2: chemotherapy must not begin earlier than one hour after the end of cetuximab.
- Observation attribution: NCCP permits stopping routine post-infusion observation after two tolerated doses; EMA Erbitux specifies close monitoring during and for at least one hour after infusion. Reconcile the approved local observation policy; do not automate removal of monitoring.
- Medication review: NCCP contraindicates concomitant brivudine, sorivudine and analogues with fluorouracil; review dihydropyrimidine dehydrogenase (a medicine-processing enzyme) (DPD)-affecting medicines, phenytoin toxicity/levels and warfarin international normalised ratio (blood-clotting test) (INR).
- Review CYP3A inducers/inhibitors and grapefruit; avoid prochlorperazine on the same day as irinotecan. Gilbert syndrome warrants consideration of a reduced starting irinotecan dose; no universal percentage is supplied.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Eyes

Cetuximab

Eye pain or new visual change needs prompt advice.

2 Mouth / swallowing

Fluorouracil

Pain or ulcers affecting eating or drinking need advice.

3 Breathing / reaction

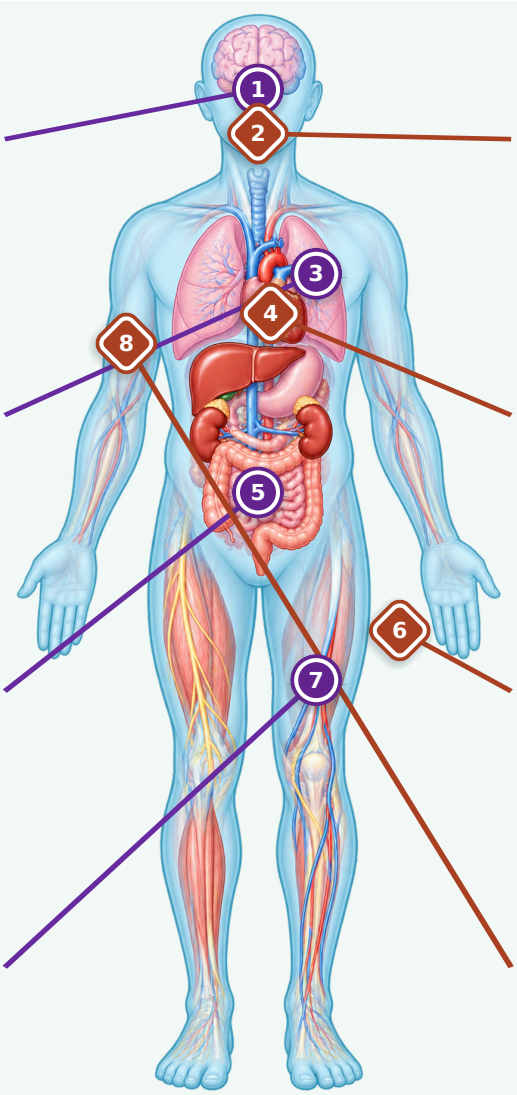
Cetuximab

New breathing difficulty requires prompt assessment.

4 Chest

Fluorouracil

New chest pain needs urgent assessment.



5 Bowel

Irinotecan

Diarrhoea, increased stoma output or dehydration: call early.

6 Skin / nails

Cetuximab

Report painful rash, cracks or infected skin early.

7 Blood / infection

Treatment

Fever, shivers or sudden illness: contact oncology immediately.

8 Line / pump

Treatment delivery

Pain, swelling, leakage or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Report eye pain, blurred vision, blistering or infected skin promptly. Call for line pain, swelling, leakage or unexpected pump progress.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Your record and contact card

IN THIS CHAPTER

Space for your own cycle or review record, questions and notes, followed by your treatment card.

Record each cycle separately. Each medicine has its own symptom table. Mark the worst symptoms since your last review, when they occurred and what they stopped you doing. Overlapping symptoms can be recorded under more than one medicine; you do not have to identify the cause.

These are patient-language severity prompts, not a validated questionnaire or a formal Common Terminology Criteria for Adverse Events (CTCAE) grading tool. Your team assigns the clinical grade. Serious symptoms override the columns: call without waiting to decide a severity level.

Use a new record sheet for each cycle or review. Copy or print another sheet when needed; the number of sheets does not prescribe the length of treatment.

Cycle number: _____ Start date: _____ Next review: _____

Cetuximab · EGFR-targeted antibody

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Skin / nail changes	□ None	□ Mild change: report	□ Pain or daily tasks affected: call	□ Spreading infection or blistering: urgent review	□ Severe illness / collapse: emergency help
Eye symptoms	□ None	□ New irritation: report	□ Pain, redness or light sensitivity: call promptly	□ Blurred vision / severe eye pain: urgent review	□ Sudden major vision loss: emergency assessment
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999

Irinotecan · Topoisomerase I chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Nausea	□ None	□ Queasy but eating and drinking	□ Intake reduced: contact the team	□ Unable to maintain intake: urgent review	□ Collapse / severe dehydration: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999

Folinic acid · Chemotherapy modulator

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999

Fluorouracil · Chemotherapy

Symptom	None	Mild	Moderate / call	Severe / urgent	Emergency signs
Diarrhoea	□ Usual pattern	□ Up to 3 extra stools above normal	□ 4–6 extra stools/ day or at night: call	□ 7+ extra stools/ day or dehydration: urgent review	□ Collapse / severe dehydration: 112/999
Mouth soreness / ulcers	□ None	□ Mild soreness; eating normally	□ Painful; soft food needed: call	□ Cannot eat / drink enough: urgent review	□ Major breathing difficulty: 112/999
Fatigue	□ Usual energy	□ Tiredness improves with rest	□ Rest does not help; usual tasks affected: call	□ Self-care difficult: urgent review	□ Collapse: 112/999
Hand-foot soreness	□ None	□ Redness or tingling without pain	□ Pain limits usual tasks: call	□ Pain limits washing / dressing: urgent review	□ Acute severe illness: emergency help

Dates / doses missed or changed / symptoms affecting daily life / questions:

Red flags override the table. Fever, shivers, suddenly unwell, chest symptoms or inability to drink: seek prompt advice. Severe breathing difficulty, collapse or severe chest pain: 112 / 999.

Keep your treatment details to hand

Advanced / metastatic colorectal cancer · NCCP 00585 v5b · Document revised 5 November 2026

Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Fortnightly cetuximab + FOLFIRI

D1 every 14 days; 46-hour fluorouracil pump to D3; no D8 visit.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record; the team confirms the total treatment plan.

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

SACTCheck v0.84.35 · NCCP 00585 v5b · Wallet-card sheet
Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

CHAPTER 05

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00585 v5b · current official regimen checked 5 November 2026**
   https://healthservice.hse.ie/documents/6395/585_v5b_Cetuximab_14_days_FOLFIRI.pdf
2. **EMA: Erbitux product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf
3. **eviQ: implanted port**
   https://www.eviq.org.au/p/507
4. **eviQ: peripherally inserted central catheter (PICC)**
   https://www.eviq.org.au/p/543
5. **eviQ: infusion pump care**
   https://www.eviq.org.au/p/502
6. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
7. **CRYSTAL extended RAS primary report**
   https://pubmed.ncbi.nlm.nih.gov/25605843/
8. **NCCP NCIS antiemetics V8 (2025), Table 1 and multi-day principles**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
9. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq